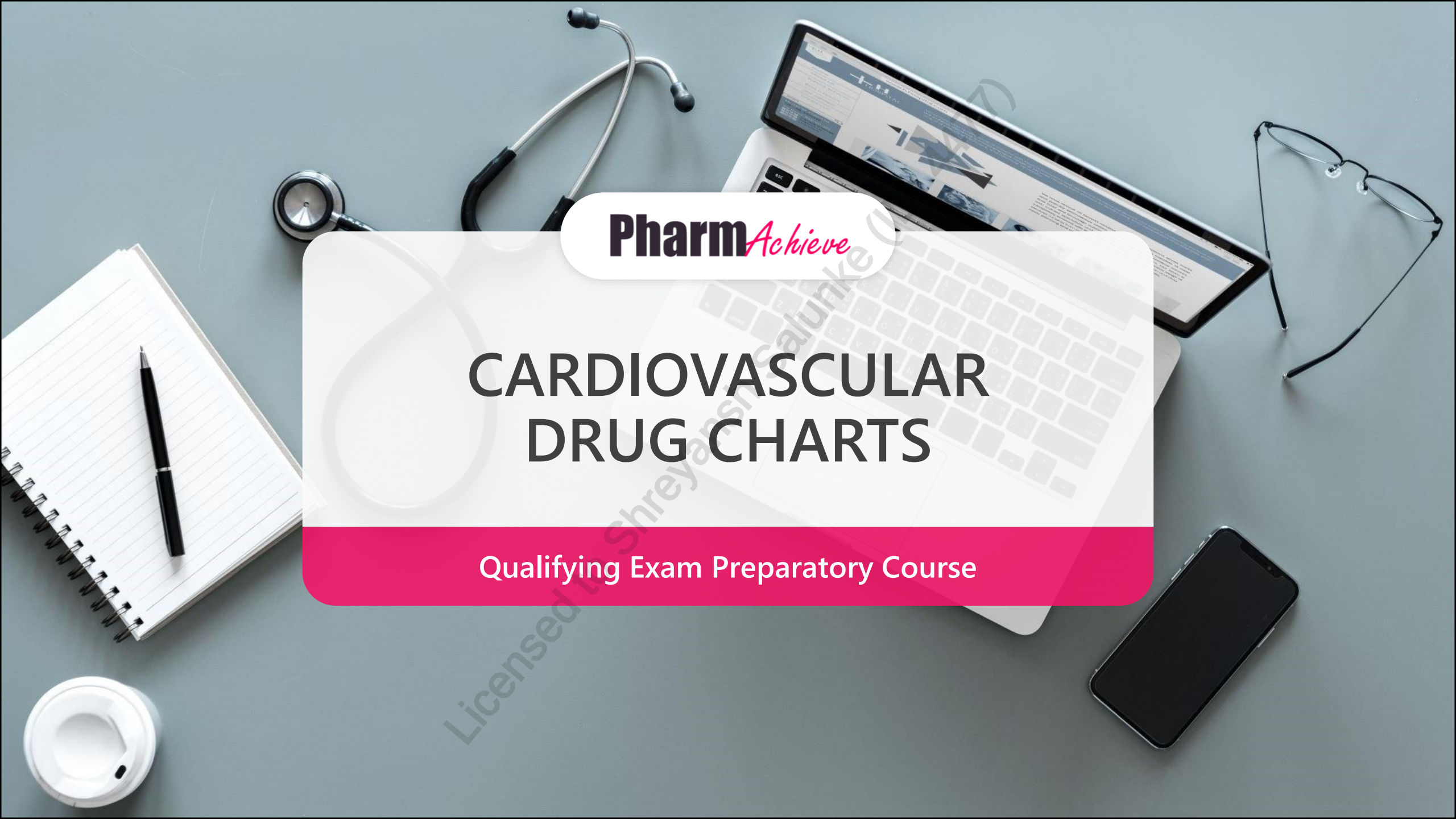




Pharm*Achieve*

CARDIOVASCULAR DRUG CHARTS

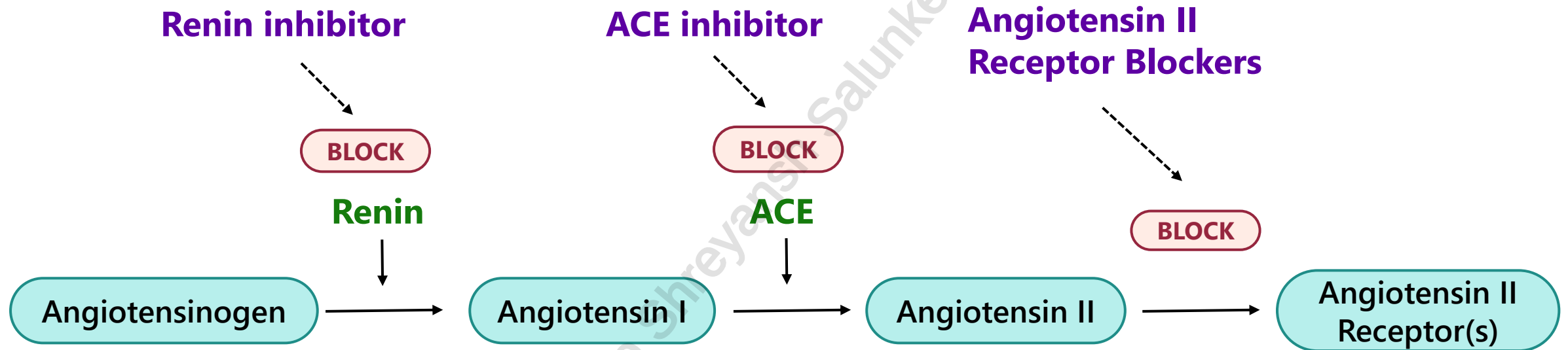
Qualifying Exam Preparatory Course

LEGEND

ACEi	Angiotensin-Converting-Enzyme Inhibitor
ADP	Adenosine Diphosphate
AE	After Effects
ARB	Angiotensin Receptor Blocker
ARNi	Angiotensin II Receptor Blocker Neprilysin Inhibitor
ASA	Acetylsalicylic Acid
BP	Blood Pressure
Ca	Calcium
CCB	Calcium Channel Blocker
CK	Creatine Kinase
Cl	Chlorine
CYP	Cytochrome P450
DHP-CCB	Dihydropyridine Calcium-Channel Blocker
D/I	Drug Interaction
GI	Gastrointestinal
HR	Heart Rate
If	Funny Current

ISA	Intrinsic Sympathomimetic Activity
K	Potassium
Li	Lithium
Mg	Magnesium
MI	Myocardial Infarction
MOA	Mechanism Of Action
MRA	Mineralocorticoid Receptor Antagonist
N/V/D	Nausea, Vomiting, Diarrhea
Na	Sodium
nDHP-CCB	Non-Dihydropyridine Calcium-Channel Blocker
NSAID	Non-Steroidal Anti-Inflammatory Drug
P2Y12	P2Y12 Protein
PCSK9	Proprotein Convertase Subtilisin Kexin Type 9
PDE5	Phosphodiesterase Type 5
P-gp	P-glycoprotein
PPARα	Peroxisome Proliferator-Activated Receptor Alpha
SGLT2	Sodium-Glucose Cotransporter-2
TdP	Torsades De Pointes

RAAS System



ANGIOTENSIN CONVERTING ENZYME INHIBITORS (ACEIs)

Drugs	MOA	Safety	Interactions
Benazepril: 10-40 mg/day Captopril: 25-150 mg/day Cilazapril: 2.5-10 mg/day Enalapril: 5-40 mg/day Fosinopril: 10-40 mg/day Lisinopril: 10-40 mg/day Perindopril: 4-8 mg/day Quinapril: 10-40 mg/day Ramipril: 2.5-20 mg/day Trandolapril: 1-4 mg/day	Inhibits angiotensin converting enzyme	- Side effects: dry cough, hypotension, increased potassium, angioedema (rare), decreased GFR (<30% is tolerable), acute kidney failure in bilateral renal artery stenosis, skin rashes, taste disturbances, proteinuria, neutropenia, dizziness, headaches - Contraindications: pregnancy, bilateral renal artery stenosis, history of angioedema	- Anti-hypertensives - K-sparing agents - NSAIDs - Li - Allopurinol

ANGIOTENSIN RECEPTOR BLOCKERS (ARBs)

Drugs	MOA	Safety	Interactions
Azilsartan: 40-80 mg/day Candesartan: 8-18 mg/day Eprosartan: 600-800 mg/day Irbesartan: 150-300 mg/day Losartan: 50-100 mg/day Olmesartan: 20-40 mg/day Telmisartan: 80 mg/day Valsartan: 80-320 mg/day	Blocks angiotensin receptor type 2	- Side effects: hypotension, headache, hyperkalemia, dizziness, decreased GFR, angioedema (rare), acute kidney failure in bilateral renal artery stenosis - Contraindications: pregnancy, bilateral renal stenosis, ARB-associated anaphylaxis or angioedema (note, no cross sensitivity with ACEI regarding angioedema)	- Anti-hypertensives - K-sparing agents - NSAIDs (fluid retention) - Lithium

BETA BLOCKERS

Drugs	MOA	Safety	Interactions
Nadolol: 20-320 mg/day Propranolol: 80-480 mg/day Timolol: 10-60 mg/day Atenolol: 25-100 mg/day Bisoprolol: 5-20 mg/day Metoprolol: 50-400 mg/day Nebivolol: 5-20 mg/day Pindolol: 10-60 mg/day Acebutolol: 100-800 mg/day Labetalol: 100-1200 mg/day	Binds to β_1/β_2 receptors and antagonizes them	- Side effects: bradycardia, fatigue, reduced exercise capacity, headaches, impotence, heart block, dysglycemia, depression, vivid dreams, heart failure, masking of hypoglycemia - Taper over 1-2 weeks to minimize risk of rebound $\uparrow$HR/BP, severe angina - Labetalol is safe in pregnancy	- Anti-hypertensives - Antiarrhythmics - Antihyperglycemics - NSAIDs - Digoxin - nDHP-CCBs

TYPES OF BETA-BLOCKERS

Non-Selective	Non-Selective + ISA Activity	B1 Selective	Selective + ISA activity	Alpha-blocking activity
• Timolol • Propranolol • Nadolol	• Pindolol	• Nebivolol • Metoprolol • Bisoprolol • Atenolol	• Acebutolol	• Labetalol
		Less non-cardiac AEs due to B1 selectivity	Less non-cardiac AEs due to B1 selectivity	Additional side effects: dizziness, edema, nasal congestion, postural hypotension

DIHYDROPYRIDINE CALCIUM CHANNEL BLOCKERS

Drugs	MOA	Safety	Interactions
Amlodipine: 2.5-10 mg/day Felodipine: 2.5-20 mg/day Nifedipine ER: 30-120 mg/day Nimodipine: 60 mg/day	Blocks voltage-gated calcium channels	• Side effects: hypotension, headache, flushing, peripheral edema, gingival hyperplasia (amlodipine), nausea (nimodipine) • Avoid short acting nifedipine because it increases risk of MI and death • Contraindications: severe hypotension, cardiogenic shock	• Strong CYP3A4 inhibitors • Anti-hypertensives

NON-DIHYDROPYRIDINE CALCIUM CHANNEL BLOCKERS

Drugs	MOA	Safety	Interactions
Diltiazem: 120-360 mg/day Verapamil: 240-480 mg/day	Blocks voltage-gated calcium channels but are more cardio-selective than DHP-CCBs	Side effects: constipation (verapamil), anorexia, dizziness, nausea, ↓BP, bradycardia, heart block, new or worsening heart failure Contraindications: ejection fraction <40%, 2 nd or 3 rd degree AV block, severe hypotension (SBP < 90 mmHg)	• Strong CYP3A4 inhibitors or inducers • Beta-blockers • antiarrhythmics (e.g. digoxin, amiodarone) • Anti-hypertensives • CYP3A4 substrates (e.g. simvastatin) • Verapamil can also increase digoxin levels

THIAZIDE DIURETICS

Drugs	MOA	Safety	Interactions
Hydrochlorothiazide: 12.5-25 mg/day Chlorthalidone: 12.5-25 mg/day Indapamide: 1.25-2.5 mg/day Metolazone: 2.5-10 mg/day	Na ⁺ -Cl ⁻ co-transporter inhibition in the kidney tubules	Side effects: diuresis, ↓K ⁺ / ↓Na ⁺ / ↓Cl ⁻ / ↓Mg ²⁺ , ↑Ca ²⁺ , hyperuricemia, hypotension, weakness and muscle cramps, hyperglycemia, hyperlipidemia, azotemia (rare), blood dyscrasias (rare)	• Lithium • Digoxin • NSAIDs • Anti-hypertensives

LOOP DIURETICS

Drugs	MOA	Safety	Interactions
Bumetanide: 1-4 mg/day Furosemide: 20-500 mg/day PO Ethacrynic acid: 50-200 mg/day	Inhibition of sodium reabsorption in the loop of Henle	Side effects: electrolyte abnormalities (e.g., hypokalemia, increased urinary frequency, dehydration, hyperuricemia, ototoxicity, hyperglycemia, fatigue, weakness, nausea, increase in total cholesterol	• NSAIDs • Corticosteroids • Aminoglycosides • Lithium • Thiazide diuretics • Digoxin

MINERALOCORTICOID RECEPTOR ANTAGONISTS (MRA)

Drugs	MOA	Safety	Interactions
Eplerenone: 25-50 mg/day Spironolactone: 12.5-50 mg/day Finerenone: 10-20 mg/day	Antagonism of aldosterone	Side effects: hyperkalemia, hypotension, dizziness, diarrhea (eplerenone), nausea, gynecomastia (spironolactone), lupus (spironolactone), agranulocytosis (spironolactone) Avoid initiating spironolactone and eplerenone in eGFR < 30 mL/min/ 1.73 m ² Avoid initiating finerenone when eGFR < 25 mL/min/ 1.73 m ²	• Anti-hypertensives • K⁺ -sparing agents • Digoxin • NSAIDs • Lithium • Calcineurin inhibitors (cyclosporine and tacrolimus)

VASODILATORS

Drugs	MOA	Safety	Interactions
Hydralazine: 30-300 mg/day	Direct acting peripheral vasodilator; ↓ peripheral vascular resistance	Side effects: rash, GI upset, hypotension, lupus-like syndrome (rare)	• Anti-hypertensives
Nitroprusside: 0.1-5 mcg/kg/min IV	Smooth muscle relaxation	Side effects: hypotension, thiocyanate and cyanide toxicity, methemoglobinemia, coronary artery steal, increased intracranial pressure, tachycardia	• Anti-hypertensives • Cyclosporine, • PDE5 inhibitors • NSAIDs
Isosorbide dinitrate: 30-120 mg/day Isosorbide mononitrate: 30-240 mg/day	Dilation of blood vessels and smooth muscle relaxation	Side effects: headache, hypotension, flushing, weakness	• Anti-hypertensives • PDE5 inhibitors (e.g. sildenafil)
Nitroglycerin 5-200 mcg/kg/min IV 0.3-0.6 mg SL PRN 0.2-0.8 mg/hr topical patch 1 spray SL PRN (pumpspray)	Dilation of blood vessels and smooth muscle relaxation	Side effects: tachycardia, bradycardia, hypotension, syncope, headache, dizziness, flushing, weakness	• Anti-hypertensives • PDE5 inhibitors (e.g. sildenafil)

PERIPHERAL ALPHA BLOCKERS

Drugs	MOA	Safety	Interactions
Doxazosin: 1-16 mg/day Prazosin: 0.5-20 mg/day Terazosin: 1-20 mg/day	Inhibitors of alpha 1 adrenergic receptors	Side effects: postural hypotension, headache, palpitations, vivid dreams, nasal congestion, palpitations, drowsiness, transient syncope (at onset of therapy)	• Anti-hypertensives • Other drugs with hypotensive properties (e.g. PDE5 inhibitors)

CENTRALLY ACTING ANTIHYPERTENSIVES

Drugs	MOA	Safety	Interactions
Methyldopa: 500-3000 mg/day	Stimulation of central alpha-adrenergic receptors	• Side effects: dry mouth, nasal congestion, depression, orthostatic hypotension, drowsiness, palpitations, water and sodium retention, sexual dysfunction • Safe in pregnancy	• Anti-hypertensives • Levodopa • Iron salts • Lithium
Clonidine: 0.2-0.6 mg/day in divided doses	Alpha agonist properties → ↓ BP	• Orthostatic hypotension and associated dizziness • Sedation • Anticholinergic side effects (e.g. dry mouth)	• Anti-hypertensives • CNS depressants • NSAIDs

SGLT2 INHIBITORS

Drug class	MOA	Safety	C/I and D/I
SGLT2 Inhibitors • Canagliflozin • Empagliflozin • Dapagliflozin ~0.5-0.7%	• Inhibits glucose reabsorption in the kidneys • Empagliflozin canagliflozin and dapagliflozin slow neuropathy progression	AEs: • UTI, yeast infection, hypotension, hyperkalemia • Weight loss (2-3 kg) • rare DKA • ↑risk of fractures and lower extremity amputations with canagliflozin in one trial • Reduced glycemic efficacy at lower eGFR	Renal Dosing (eGFR ml/min/1.73 m³): • Dose reduction considered in renal impairment • Dapagliflozin – eGFR <25: do not start but may continue • Canagliflozin- if eGFR <60: reduce dose. If eGFR <30, do not start but may continue • Empagliflozin - if eGFR <30: reduce dose. If eGFR <20: do not start but can continue • Canagliflozin: Strong enzyme inducers (e.g. carbamazepine, rifampin, phenytoin) • Loop diuretics ↑ risk of hypotension

IF INHIBITOR

Drugs	MOA	Safety	Interactions
Ivabradine: 5-15 mg/day	Inhibition of If current which results in a decrease in heart rate	- Side effects: bradycardia, atrial fibrillation, visual effects Must have HR ≥ 70 bpm (Health Canada-approved indication states a resting HR of ≥ 77 bpm) - Contraindications: sick sinus syndrome, 3rd degree heart block, non-DHP CCB	- Moderate- Strong CYP3A4 inhibitors/inducers - QT prolonging drugs (AVOID unless necessary) - Potassium depleting agents - HR lowering agents

ARNI AND POSITIVE INOTROPES

Drug Class	MOA	Safety	Interactions
ANGIOTENSIN RECEPTOR-NEPRILYSIN INHIBITOR (ARNI): Valsartan/sacubitril: 51/49 mg –103/97 mg BID	Valsartan: See ARB MOA Sacubitril: pro drug →Neprilysin inhibition	Side effects: hypotension, renal insufficiency, hyperkalemia, should not be used in pregnancy	• Anti-hypertensives • K-sparing agents • NSAIDs • Li
POSITIVE INOTROPES: Milrinone: 0.1-0.75 mcg/kg/min IV Dobutamine: 2-20 mcg/kg/min IV	Phosphodiesterase III inhibitor (milrinone) and α/β -adrenergic agonist properties (dobutamine)	Side effects: tachycardia, cardiac/peripheral ischemia, hypotension, thrombocytopenia (milrinone)	• Beta-blockers (dobutamine)

CARDIAC GLYCOSIDE

Drugs	MOA	Safety	Interactions
Digoxin: 0.0625-0.25 mg/day	Inhibits Na ⁺ -K ⁺ ATPase	Side effects: GI (N/V/D anorexia), fatigue, dizziness, bradycardia, AV block, may cause arrhythmias, confusion, delirium, visual disturbances (blurred or yellow vision)	• Antiarrhythmics/rate control agents • Diuretics • Dose adjust in renal impairment • Hyperthyroidism • CYP3A4 inhibitors (e.g. clarithromycin) & inducers (e.g. St. John's Wort) • Antacids, cholestyramine, colestipol

SOLUBLE GUANYLATE CYCLASE STIMULATOR

Drug	MOA	Safety	Interactions
Vericiguat	Stimulates soluble guanylate cyclase (sGC) to increase intracellular cyclic guanosine monophosphate (cGMP). This improves vasodilation and smooth muscle relaxation	Side effects: hypotension, dyspepsia, nausea, anemia	PDE5 inhibitor (sildenafil)

HMG CO-A REDUCTASE INHIBITORS

Drugs	MOA	Safety	Interactions
Rosuvastatin: 10-40 mg/day; CYP2C9 substrate	Inhibits HMG CoA reductase	- Side effects: myalgia, increased CK and transaminases, rash, sleep disturbances, headache, dizziness, GI effects, rhabdomyolysis (rare), lupus (rare), peripheral neuropathy (rare), impotency, myopathy - Contraindications: pregnancy, active liver disease	- Fibrate - CYP450 interactions
Simvastatin: 10-80 mg/day; CYP3A4 substrate			
Atorvastatin: 10-80 mg/day; CYP3A4 substrate			
Fluvastatin: 20-80 mg/day; CYP2C9 substrate			
Pravastatin: 10-40 mg/day; No CYP D/I			
Lovastatin: 20-80 mg/day; CYP3A4 substrate			

EZETIMIBE

Drugs	MOA	Safety	Interactions
Ezetimibe: 10 mg/day	Inhibits intestinal cholesterol absorption	Side effects: abdominal pain, diarrhea, back pain, arthralgias, upper respiratory tract infection (URTI), fatigue, dizziness, headache, myopathy (rare), hepatitis (rare), rhabdomyolysis (rare), thrombocytopenia (rare), acute pancreatitis (rare)	• Cyclosporine • Cholestyramine

NIACIN

Drugs	MOA	Safety	Interactions
Niacin: 50 mg TID	↓ Esterification of hepatic triglycerides, LDL, VLDL, ↑ HDL	Side effects: flushing, pruritus, dry skin, acanthosis nigricans, burning, stinging, N/V/D, dyspepsia, increased blood sugars, myotoxicity, hepatotoxicity (rare), TdP (rare), increased uric acid levels	Statins

FIBRATES

Drugs	MOA	Safety	Interactions
Bezafibrate: 400 mg/day Fenofibrate: 200-400 mg/day Gemfibrozil: 300-1200 mg/day	Increased activity of PPAR α (peroxisome proliferator-activated receptor-alpha) which results in an increase in lipid metabolism	- Side effects: myalgia, myositis, rhabdomyolysis (with gemfibrozil), N/V/D, abdominal pain, increased glucose, increased CK, increased bile lithogenicity, increased serum creatinine - Contraindications: severe hepatic and renal dysfunction, gall bladder disease	- Statins - Warfarin - Ezetimibe

PCSK9 INHIBITORS

Drugs	MOA	Safety	Interactions
Evolocumab: 140-240 mg SC monthly Alirocumab: 75-150 mg SC every 2 weeks	Inhibits PCSK9 to ↑uptake of cholesterol	Side effects: injection site reactions, hypersensitivity, auto-antibody formation, upper respiratory tract infections	No formal drug-drug interactions have been studied
Inclisiran sodium: 284 mg SC First dose followed by second dose at 3 months Subsequent doses every 6 months	Small interfering ribonucleic acid (siRNA) that degrades proprotein convertase subtilisin kexin type 9 (PCSK9) mRNA	Side effects: injection site reaction/pain/rash,	No formal drug-drug interactions have been studied

RESINS

Drugs	MOA	Safety	Interactions
Cholestyramine: 4-24 g/day Colesevelam: Powder: 3.75 g/day Tablets: 2.5 g/day Colestipol: Granules: 10-15 g/day Tablets: 2-4 g/day	Binds to bile acids in the GI tract	Side effects: constipation, abdominal fullness and flatulence, bloating, increased triglycerides, increased hyperchloremic acidosis, pancreatitis, cholelithiasis, cholecystitis, malabsorption syndrome, GI bleeding, peptic ulceration	Give other medications 1 hour before or 4-6 hours after the resin medication

FIBRINOLYTIC AGENTS

Drugs	MOA	Safety	Interactions
Alteplase: 0.9 mg/kg IV Tenecteplase: 30-50 mg IV	Converts plasminogen to plasmin in the presence of fibrin, resulting in fibrinolysis	Side effects: bleeding, cerebral edema and herniation, seizures, new-onset ischemic stroke	The interaction of recombinant alteplase with other drugs has not been formally studied

P2Y12 INHIBITORS

Drugs	MOA	Safety	Interactions
Clopidogrel: 75 mg/day	- Prodrug metabolized into its active form through CYP2C19 - Irreversibly inhibits P2Y12 ADP receptor on platelets	Side effects: diarrhea, rash, purpura, bleeding, neutropenia (rare), thrombocytopenic purpura (rare)	- CYP2C19 inhibitors (e.g. omeprazole and esomeprazole) - Antiplatelet agents - Anticoagulants
Ticagrelor: 120 mg/day	Reversible inhibition of the P2Y12 ADP receptor	Side effects: bleeding, dyspnea, bradycardia	- Antiplatelet agents - Anticoagulants
Prasugrel: 10 mg/day	- Prodrug metabolized into its active form through CYP3A4, CYP2B6 - Irreversibly inhibits P2Y12 ADP receptor on platelets	Side effects: bleeding, rash, headache, dizziness, hypo/hypertension, hypercholesterolemia CI if history of stroke. Avoid if <60 kg or >75 years old	- Antiplatelet agents - Anticoagulants - CYP3A4 inhibitors

VITAMIN K ANTAGONIST

Drugs	MOA	Safety	Interactions
Warfarin: Dosing based on INR	Inhibits the production of vitamin K-dependent clotting factor	Side effects: bleeding, skin necrosis, skin necrosis, bruising, purple toes syndrome	• Antibiotics • Tylenol® • Anti-platelets/ anticoagulants • Vitamin K-rich foods • CYP2C9 interactions

FACTOR XA INHIBITORS

Drugs	MOA	Safety	Interactions
Fondaparinux: 2.5 mg daily Apixaban: 5-10 mg/day Edoxaban: 60 mg/day Rivaroxaban: 20 mg/day	Inhibition of factor Xa by binding to anti-thrombin	Side effects: bleeding, allergic reactions	• Drugs which may impact hemostasis • CYP3A4 and P-gp interactions (apixaban, edoxaban, rivaroxaban)

DIRECT THROMBIN INHIBITORS

Drugs	MOA	Safety	Interactions
Bivalirudin: 1.75 mg/kg/h Dabigatran: 220-300 mg/day	Inhibition of thrombin	• Dabigatran: GI upset • Side effects: bleeding, allergic reactions	• Drugs which affect hemostasis • P-glycoprotein interactions (dabigatran) • PPIs (dabigatran)

ASA

Drug Class	MOA	Safety	Interactions
ASA: 80-325 mg/day	Inhibits platelet aggregation by irreversibly inhibiting thromboxane A2 synthesis	Side effects: bleeding (rare at low doses), rash, nausea, dyspepsia, N/V, peptic ulcer disease (PUD), tinnitus, vertigo, bronchospasm (rare)	• NSAIDs • Antiplatelet agents • Anticoagulants

GLYCOPROTEIN IIB/IIIA INHIBITORS

Drugs	MOA	Safety	Interactions
Eptifibatide: 2 mcg/kg/h Tirofiban: 0.1 mcg/kg/h	Prevents ligands from binding to glycoprotein IIb/IIIa which causes inhibition of platelet aggregation	• Bleeding risk • Allergic reactions • Thrombocytopenia	Drugs which impact hemostasis Drugs that ↑ bleeding risk

HEPARINS

Drugs	MOA	Safety	Interactions
Unfractionated heparin 60-70 units/kg	Potentiates antithrombin, inhibits factor Xa and IIa	Side effects: bleeding, hematoma, heparin induced thrombocytopenia (HIT), thrombocytopenia, hypersensitivity reactions	Drugs that ↑ bleeding risk
Low molecular weight heparin Dalteparin: 120 units/kg/day Enoxaparin: 1 mg/kg/day BID	Enhances the action of antithrombin leading to a reduction in clot formation		

CLASS 1A ANTI-ARRHYTHMIC

Drugs	MOA	Safety	Interactions
Procainamide: 15-17 mg/kg IV	Decreased conduction from inhibition of the sodium channel	Side effects: TdP, hypotension	QT prolonging drugs

CLASS 1C ANTI-ARRHYTHMICS

Drugs	MOA	Safety	Interactions
Flecainide: 100-400 mg/day	Alters transport of ions across membranes leading to slower conduction	AEs: Tremors, blurry vision, heart failure, ventricular tachycardia, pro-arrhythmia	CYP2D6 inhibitors, other antiarrhythmics, QT prolonging agents
Propafenone: 150-2400 mg/day		AEs: Ventricular tachycardia pro-arrhythmia, headache, constipation, metallic taste	CYP2D6 interactions, other antiarrhythmics, QT prolonging agents, digoxin

CLASS III ANTI-ARRHYTHMICS

Drugs	MOA	Safety	Interactions
Amiodarone: 200 mg TID PO	Inhibition of potassium, calcium and sodium channels	Side effects: GI upset, dermatologic effects, neurologic effects, ophthalmologic effects, thyroid effects, pulmonary fibrosis, liver dysfunction, aggravation of arrhythmias (rare)	• Warfarin • Beta-blockers • Digoxin • Anti-arrhythmic drugs • P-gp and CYP interactions (e.g. CYP3A4)
Dronedarone: 400 mg BID		Side effects: N/V/D, dyspepsia, fatigue/weakness, liver dysfunction	• CYP3A4, 2D6 and P-gp interactions • QT prolonging agents

CLASS III ANTI-ARRHYTHMICS

Drugs	MOA	Safety	Interactions
Ibutilide: 0.01-0.1 mg/kg IV	Inhibition of potassium channels and activates a slow, delayed inward sodium current	Side effects: TdP, hypotension	QT prolonging medications
Sotalol 80-240 mg BID	Inhibition of potassium channels and beta-adrenoreceptor-blocking properties ➤ Also considered class II/III anti-arrhythmic	Side effects: hypotension, bradycardia, ventricular tachycardia pro-arrhythmia, TdP	CYP2D6 interactions, other antiarrhythmics, QT prolonging agents , digoxin

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CHANGE LOG

February 2025

- Slide 3: Updated legend
- Slide 20: New added vericiguat
- Slide 29: Removed acenocoumarol as discontinued
- Slide 32: Removed Aggrenox® as brand and generic discontinued

February 2025

- Slide 2: Updated legend
- Slide 6: Removed aliskiren as discontinued

May 2025

- Minor formatting changes made throughout
- Slide 9: Added contraindications
- Slide 12: Added interaction with calcineurin inhibitors
- Slide 15: Added clonidine
- Slide 17: Changed heart rate threshold to ≥ 70 bpm and added note on Health Canada indication
- Slides 28, 37 & 38: Edited and added information

June 2025

- Slide 16: Added SGLT2 drug table

October 2025

- Slide 32: Elaborated on the MOA of ASA

January 2026

- Slide 12: Renamed drug class MRA, added finerenone, and added renal considerations
- Slide 16: Updated SGLT2 inhibitor thresholds for renal adjustment